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Marks	0/63
Grade	0 out of 100



Question 1

Not answered

Marked out of 1

In which of the following age groups is the prevalence of depression in males equivalent to that in females?

Select one:

- ☐ Middle aged adults
- ☐ Adolescents
- ☐ Prepubertal children
- ☐ Older adults
- ☐ Young adults

Your answer is incorrect.

Explanation:

The prevalence of depression in pre-pubertal children is approximately 1-2%, with no sex difference. By mid-teens, the prevalence is around 4-5%, with the female predominance characteristic of adult depression clearly established.

Ref: Maughan B et al. Depression in childhood and adolescence. Journal of the Canadian Academy of Child and Adolescent Psychiatry. 2013 Feb;22(1):35.

The correct answer is: Prepubertal children

Question 2

Not answered

Marked out of 1

Which mode of treatment is the mainstay in the management of encopresis?

Select one:

- ☐ CBT
- ☐ Treatment with Tricyclic antidepressants
- ☐ Treatment with SSRIs
- ☐ Behavioural modification
- ☐ Family Therapy

Your answer is incorrect.

In most cases of encopresis, considerable family discord and distress are common. It is important to reduce the family tensions about the symptom and a nonpunitive atmosphere is established. Education of the family and correction of misperceptions that a family may have about soiling must occur before treatment. A paediatric referral is always indicated. Behavioural methods are the mainstay of treatment and ongoing behavioural intervention to enhance appropriate bowel behaviour and diminish anxiety related to bowel movement.

The correct answer is: Behavioural modification

Question 3

Not answered

Marked out of 1

What genetic aetiology most increases the risk of parents having a second child with Down syndrome?

Select one:

- ☐ Trisomy 18
- ☐ Mosaicism
- ☐ Trisomy 23
- ☐ Translocation
- ☐ Trisomy 21

Your answer is incorrect.

Explanation:

In translocation, a fusion occurs of two chromosomes, usually 21 and 15, resulting in a total of 46 chromosomes, despite the presence of one extra chromosome 21. Unlike trisomy 21, this is usually inherited, and the translocated chromosome may be found in unaffected parents or siblings (who only have 45 chromosomes).

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 101.

The correct answer is: Translocation

Question 4

Not answered

Marked out of 1

Which one of the following is not recommended in the management of anxiety disorders in children and adolescents?

Select one:

- ☐ SSRIs
- ☐ CBT
- ☐ Benzodiazepines
- ☐ Psychoeducation
- ☐ Family and play therapy

Your answer is incorrect.

Explanation:

For the treatment of anxiety disorders in children and adolescents, psychoeducation can help families understand the condition and provide reassurance. CBT has been proven to be effective, incorporating both cognitive (e.g. reframing, positive self-talk, challenging unhelpful thoughts, and weighing up evidence for and against expected events) and behavioural processes (e.g. systematic desensitisation, exposure and response prevention for specific phobias, relaxation training, modelling, rewarding wanted behaviour, and role-play). Family therapy may help when dysfunctional patterns of family interaction influence the child's anxiety symptoms. SSRIs can be effective and are generally well-tolerated. Medication is usually most considered in older children with more severe symptoms. Benzodiazepines have NOT shown efficacy; their side effects (e.g. behavioural disinhibition) are a risk in this population.

Ref: Vallance AK, Fernandez V. Anxiety disorders in children and adolescents: aetiology, diagnosis and treatment. BJPsych Advances. 2016 Sep;22(5):335-44.

The correct answer is: Benzodiazepines

Question 5

Not answered

Marked out of 1

The rate of self-harm in teenagers who identify with Goth culture is around

Select one:

- ☐ 90%
- ☐ 1%
- ☐ 50%
- ☐ 7%
- ☐ 15%

Your answer is incorrect.

Explanation:

About half of 'Alternative' (e.g. Goth) adolescents self-injure, primarily to regulate emotions and communicate distress.

Ref: Young R, Sproeber N, Groschwitz RC, Preiss M, Plener PL. Why alternative teenagers self-harm: exploring the link between non-suicidal self-injury, attempted suicide and adolescent identity. BMC psychiatry. 2014 Dec 1;14(1):137.

The correct answer is: 50%



Question 6

Not answered

Marked out of 1

In a child with developmental disturbances, magnetic resonance imaging (MRI) reveals hypoplasia of the cerebellar vermis. The diagnosis is most likely to be

Select one:

- ☐ Passive smoking exposure
- ☐ ADHD
- ☐ Severe anxiety disorder
- ☐ Oppositional defiant disorder
- ☐ Autism spectrum disorder

Your answer is incorrect.

Explanation:

Children with cerebellar vermal agenesis or diffuse cerebellar hypoplasia commonly present with core autistic spectrum disorder symptoms including language deficits, social interaction impairments, and repetitive and stereotyped behaviours.

Ref: Hampson DR, Blatt GJ. Autism spectrum disorders and neuropathology of the cerebellum. *Frontiers in neuroscience*. 2015 Nov 6;9:420.

The correct answer is: Autism spectrum disorder

Question 7

Not answered

Marked out of 1

Tourette syndrome is characterised by all except

Select one:

- ☐ Symptoms present for at least a year
- ☐ Mean age of onset = seven years
- ☐ Multiple motor tics
- ☐ Seen more commonly in girls than boys
- ☐ One or more vocal tics

Your answer is incorrect.

Explanation:

Tourette syndrome is a complex neuropsychiatric disorder with childhood onset characterised by multiple motor and one or more vocal tics present for at least a year. Males are more commonly affected (3-4:1, male:female). The age of onset is 2-21 years, with a mean age of 7 years.

Ref: Robertson MM. Tourette syndrome in children and adolescents: aetiology, presentation and treatment. BJPsych Advances. 2016 May;22(3):165-75.

Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 587.

The correct answer is: Seen more commonly in girls than boys

Question 8

Not answered

Marked out of 1

Which of the following genetic disorders is most closely associated with autism?

Select one:

- ☐ Fragile X syndrome
- ☐ Cornelia de Lange syndrome
- ☐ Hurler syndrome
- ☐ Huntington chorea
- ☐ Prader-Willi syndrome

Your answer is incorrect.

Explanation:

Fragile X syndrome is the most common inherited cause of intellectual disability (ID). It accounts for 10-12% of ID in men. Mild ID is often seen in affected women whereas moderate/severe ID is seen in affected men. There are high rates of ADHD and autism spectrum disorder in Fragile X syndrome.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 814.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 102.

The correct answer is: Fragile X syndrome

Question 9

Not answered

Marked out of 1

In terms of treatments for children and adolescents with anorexia nervosa, which of the following treatments does a large body of evidence support?

Select one:

- ☐ Art therapy
- ☐ Cognitive analytical therapy
- ☐ Family therapy
- ☐ Cognitive behavioural therapy
- ☐ Interpersonal psychotherapy

Your answer is incorrect.

Family interventions that directly address the eating disorder are especially useful for children and adolescents with anorexia nervosa. (NICE Guidelines)

The correct answer is: Family therapy

Question 10

Not answered

Marked out of 1

The male to female ratio for conduct disorder is

Select one:

- ☐ 1 to 4
- ☐ 2 to 1
- ☐ 3 to 1
- ☐ 10 to 1
- ☐ 4 to 1

Your answer is incorrect.

Estimated rates of conduct disorder among the general population range from 1 to 10 percent, with a general population rate of approximately 5 percent. The disorder is more common among boys than girls, and the male: female ratio is 4:1 (DSM-IV TR).

The correct answer is: 4 to 1



Question **11**

Not answered

Marked out of 1

In ICD-11, enuresis is defined as the repeated involuntary or voluntary voiding of urine into clothes or bed for a child of age

Select one:

- ☐ 10 years or older
- ☐ 5 years or older
- ☐ 7 years or older
- ☐ 2 years or older
- ☐ 3 years or older

Your answer is incorrect.

Explanation:

In ICD-11, enuresis is defined as the repeated voiding of urine into clothes or bed, which may occur during the day or at night, in an individual who has reached a developmental age when urinary continence is ordinarily expected (5 years). Voiding of urine is typically involuntary but, in some cases, may appear to be voluntary. The diagnosis can be assigned in either case.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

The correct answer is: 5 years or older



Question 12

Not answered

Marked out of 1

Which of the following statements about childhood elimination disorders is correct?

Select one:

- ☐ Star charts are helpful in encopresis but not in enuresis
- ☐ Secondary enuresis is an atypical extension of normal infantile incontinence
- ☐ Nocturnal enuresis occurs more commonly in girls than in boys
- ☐ Functional encopresis is unrelated to sleep disorders
- ☐ Enuresis may occur in association with a mental disorder or psychosocial stressor

Your answer is incorrect.

Explanation:

In ICD-11, enuresis is defined as the repeated voiding of urine into clothes or bed, which may occur during the day or at night, in an individual who has reached a developmental age when urinary continence is ordinarily expected (5 years). The urinary incontinence may have been present from birth (i.e., an atypical extension of normal infantile incontinence), or may have arisen following a period of acquired bladder control ('secondary' or 'previously dry' enuresis). The common age of onset for children who have previously acquired urinary continence, yet develop enuresis, is between the ages of 5 to 8 years. Nocturnal enuresis is more common in males, whereas diurnal enuresis is more common in females. Enuresis may be an aspect of another mental, behavioural, or neurodevelopmental disorder. Psychosocial stressors appear to precipitate enuresis in a subgroup of children with the disorder. In young children, the disorder may relate to the birth of a sibling, hospitalisation, the start of school, separation of a family due to divorce, or a move to a new environment.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 680.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 485.

The correct answer is: Enuresis may occur in association with a mental disorder or psychosocial stressor

Question 13

Not answered

Marked out of 1

Which of the following is true with respect to enuresis?

Select one:

- ☐ It is always accompanied by encopresis
- ☐ Diurnal enuresis is more common in boys than girls
- ☐ It cannot be diagnosed without anatomical scan of urinary tract
- ☐ It can run in families
- ☐ It does not occur as an aspect of a mental disorder

Your answer is incorrect.

Explanation:

In ICD-11, enuresis is defined as the repeated voiding of urine into clothes or bed, which may occur during the day or at night, in an individual who has reached a developmental age when urinary continence is ordinarily expected (5 years). Nocturnal enuresis is more common in males, whereas diurnal enuresis is more common in females. Enuresis is more common among children with a parent who has a history of enuresis. Enuresis may be an aspect of another mental, behavioural, or neurodevelopmental disorder. Psychosocial stressors appear to precipitate enuresis in a subgroup of children with the disorder. In young children, the disorder may relate to the birth of a sibling, hospitalisation, the start of school, separation of a family due to divorce, or a move to a new environment.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 680.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 485.

The correct answer is: It can run in families

Question 14

Not answered

Marked out of 1

Early-onset schizophrenia is associated with all except

Select one:

- ☐ Lower IQ
- ☐ Increased social deficits in adulthood
- ☐ More cognitive deficits
- ☐ Decreased negative symptoms
- ☐ Poorer long term outcome

Your answer is incorrect.

Early-onset schizophrenia appears to be a more malignant type of schizophrenia, which presents a greater challenge to treat with greater negative symptoms and cognitive deficits. It seems to respond less to medication than schizophrenia with adult onset, and the prognosis may be poorer. Premorbid course EOS cases have increased language, motor, and social delays than adult onset cases. Recent reports have documented marked neuropsychological deficits in attention, working memory, and premorbid IQ among children who develop schizophrenia and its spectrum disorders.

The correct answer is: Decreased negative symptoms

Question 15

Not answered

Marked out of 1

What is the strongest risk factor for criminality at age 17 in a boy who has behavioural problems at the age of 8?

Select one:

- ☐ ADHD
- ☐ Harsh, punitive parenting
- ☐ Poor educational achievement
- ☐ Having no siblings
- ☐ Low IQ

Your answer is incorrect.

Explanation:

Harsh, punitive parenting characterised by severe physical and verbal aggression is associated with the development of aggressive behaviours. Chaotic home conditions are associated with delinquency. Assaultive behaviour in childhood and parental criminality predict a high risk for incarceration later in life.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 192-193.

The correct answer is: Harsh, punitive parenting



Question 16

Not answered

Marked out of 1

Clinging and diffuse non-selectively focused attachment behaviour is seen in

Select one:

- ☐ Childhood emotional disorder
- ☐ Sibling rivalry disorder
- ☐ Disinhibited attachment disorder
- ☐ Childhood disorder of social functioning
- ☐ Reactive attachment disorder

Your answer is incorrect.

Disinhibited attachment disorder of childhood is characterised by a particular pattern of abnormal social functioning that arises during the first 5 years of life. In the early stages, it is usually manifest by clinging and diffuse non-selectively focused attachment behaviour. By the age of 4 years, diffuse attachment remains but clinging tends to be replaced by attention seeking and indiscriminately friendly behaviour. During middle and later childhood this behaviour often persists and depending on circumstances, there may also be associated emotional or behavioural disturbance.

The correct answer is: Disinhibited attachment disorder

Question 17

Not answered

Marked out of 1

Separation anxiety disorder (SAD) is characterised by developmentally inappropriate and excessive anxiety concerning separation from home or from those to whom the individual is attached. This condition occurs in about

Select one:

- ☐ 0.5% of children
- ☐ 7.5 % of children
- ☐ 1.5 % of children
- ☐ 10% of children
- ☐ 3.5% of children

Your answer is incorrect.

Explanation:

Separation anxiety disorder is characterised by increased and inappropriate anxiety around separation from attachment figures or home, which is developmentally abnormal and results in impaired functioning. It occurs in about 3.5% of children and 0.8% of adolescents. Symptoms include anxiety about actual or anticipated separation from, or danger to, attachment figures; sleep disturbances and nightmares; somatisation; and school refusal.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 684.

The correct answer is: 3.5% of children



Question **18**

Not answered

Marked out of 1

According to DSM-5 criteria, the core symptoms of ADHD must have their onset before the age of

Select one:

- ☐ 9 years
- ☐ 7 years
- ☐ 12 years
- ☐ 3 years
- ☐ 18 years

Your answer is incorrect.

Explanation:

DSM-5 criteria state that several inattentive or hyperactive-impulsive symptoms must be present prior to the age of 12 years, and allows for the diagnosis in adults.

Ref: American Psychiatric Association. Diagnostic and statistical manual of mental disorders (DSM-5®). American Psychiatric Pub; 2013 May 22.

Sample D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 668.

The correct answer is: 12 years

Question 19

Not answered

Marked out of 1

Which one of the following statements about transient tic disorder is incorrect?

Select one:

- ☐ The tics occur many times a day, nearly every day for at least 4 weeks
- ☐ Quite common in middle childhood, especially among boys.
- ☐ The tics should last longer than 12 consecutive months
- ☐ The mean age of onset is between 4-7.
- ☐ The disturbance is not due to the direct physiological effects of a substance (stimulants) or a general medical condition (Huntington's disease or post viral encephalitis)

Your answer is incorrect.

Diagnostic criteria for Transient tic disorders suggest that the tics occur many times a day, nearly every day for at least 4 weeks, but for no longer than 12 consecutive months.

The correct answer is: The tics should last longer than 12 consecutive months

Question 20

Not answered

Marked out of 1

Which of the following positive symptoms is most commonly found in young people with schizophrenia is

Select one:

- ☐ Visual hallucinations
- ☐ Disorganisation
- ☐ Olfactory hallucinations
- ☐ Auditory hallucinations
- ☐ Delusions

Your answer is incorrect.

Delusions are extremely common in schizophrenia, occurring in more than 90% of those who have the disorder. Often, these delusions involve illogical or bizarre ideas or fantasies. Common schizophrenic delusions include Delusions of persecution, Delusions of reference, Delusions of grandeur and Delusions of control.

The correct answer is: Delusions

Question **21**

Not answered

Marked out of 1

An adolescent patient diagnosed with ADHD is found to have a co-morbid bipolar disorder. All of the following symptoms could be explained by either diagnosis except

Select one:

- ☐ Decreased need for sleep
- ☐ Hyperactivity
- ☐ Mood lability
- ☐ Restlessness
- ☐ Impulsivity

Your answer is incorrect.

Explanation:

ADHD can co-occur with mood disorders, such as bipolar disorder, while inattention, hyperactivity, and impulsivity can also be features of mood disorders in individuals without ADHD. Symptoms such as restlessness, pacing, and impaired concentration can be features of a depressive episode. Inattention, impulsivity, and hyperactivity are typical features of manic and hypomanic episodes. At the same time, mood lability and irritability may be associated features of ADHD.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

The correct answer is: Decreased need for sleep

Question **22**

Not answered

Marked out of 1

With respect to the delayed development of empathy skills which of the following is correct?

Select one:

- ☐ Most delayed in hearing children of hearing parents
- ☐ Most delayed in adopted children of deaf parents
- ☐ Most delayed in hearing children of hearing parents
- ☐ Most delayed in deaf children of hearing parents
- ☐ Most delayed in deaf children of deaf parents

Your answer is incorrect.

Deaf preadolescents have more difficulty with empathy development than hearing children, and this ability is related to the onset of deafness (Bachara, 1980). Deaf children born to deaf parents are better in the following areas compared to those born to hearing parents: 1. maturity 2. independence 3. responsibility taking 4. impulse control 5. empathizing ability 6. better match of parental expectations and achievements. Bachara, GH. Empathy development in deaf preadolescents. American Annals of the Deaf, 125; 38-41 Feb 1980. Moores, DF and Meadow-Orlans, KP (ed). Educational and Developmental Aspects of Deafness. Gallaudet University Press 1990. Page 291-93

The correct answer is: Most delayed in deaf children of hearing parents

Question **23**

Not answered

Marked out of 1

Of the following, the condition most commonly co-morbid with autism spectrum disorders is

Select one:

- ☐ Schizophrenia
- ☐ Conduct disorder
- ☐ Tourette syndrome
- ☐ Down syndrome
- ☐ ADHD

Your answer is incorrect.

Explanation:

Frequent co-morbidities in autism spectrum disorders are intellectual disability (80%), anxiety disorders (40-84%), ADHD (30-50%), OCD, and tic disorders.

Ref: Santosh PJ, Singh J. Drug treatment of autism spectrum disorder and its comorbidities in children and adolescents. BJPsych Advances. 2016 May;22(3):151-61.

Sample D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 674.

Leitner Y. The co-occurrence of autism and attention deficit hyperactivity disorder in children—what do we know?. Frontiers in human neuroscience. 2014 Apr 29;8:268.

The correct answer is: ADHD

Question **24**

Not answered

Marked out of 1

What is the chance of a 12-year-old boy with conduct disorder developing antisocial personality disorder?

Select one:

- ☐ 50%
- ☐ 10%
- ☐ 20%
- ☐ 40%
- ☐ 0.7%

Your answer is incorrect.

Explanation:

Around half of children with conduct disorder will receive a diagnosis of antisocial personality disorder as adults. Substance misuse is also commonly seen in adult life. There is an increased risk of early death, often by violent and sudden means.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 665.

The correct answer is: 50%



Question 25

Not answered

Marked out of 1

A 12 yr old boy is found to be skipping school repeatedly and staying out at night to use drugs with his friends. He is most likely to have which of the following psychiatric conditions?

Select one:

- ☐ Oppositional defiant disorder
- ☐ Conduct disorder
- ☐ Asperger syndrome
- ☐ ADHD
- ☐ Autism

Your answer is incorrect.

Explanation:

Conduct disorder is a repetitive and persistent pattern of antisocial, aggressive, or defiant behaviours that defy age-appropriate societal norms or rules. Children with conduct disorder are likely to demonstrate behaviours in the following four categories:

- o Aggression to people and animals (e.g. bullies, initiates fights, uses a weapon, physical cruelty or people or animals, mugging/robbery, forcing someone into sexual activity)
- o Destruction of property (e.g. fire setting or other methods of destruction)
- o Deceitfulness or theft (e.g. breaking into houses/cars, conning others, shoplifting/forgery)
- o Serious violations of rules (e.g. staying out at night, running away, school truancy)

Ref: American Psychiatric Association. Diagnostic and statistical manual of mental disorders (DSM-5®). American Psychiatric Pub; 2013 May 22.

The correct answer is: Conduct disorder

Question **26**

Not answered

Marked out of 1

You are seeing a 2-year-old boy in the clinic whose vocabulary is limited to only four words (mama, daddy, hi, and more). He rarely communicates but uses these words one at a time in appropriate situations. He supplements his infrequent verbal communications with pointing and other simple gestures to request desired objects or actions. What would be the most likely diagnosis?

Select one:

- ☐ Developmental learning disorder with impairment in mathematics
- ☐ Developmental learning disorder with impairment in reading
- ☐ Developmental language disorder with impairment of receptive and expressive language
- ☐ Developmental language disorder with impairment of mainly expressive language
- ☐ Developmental learning disorder with impairment in written expression

Your answer is incorrect.

Explanation:

Developmental language disorder with impairment of mainly expressive language is characterised by persistent difficulties in the acquisition, production, and use of language that arise during the developmental period, typically during early childhood, and cause significant limitations in the individual's ability to communicate. The ability to produce and use spoken or signed language (i.e., expressive language) is markedly below the expected level given the individual's age and level of intellectual functioning, but the ability to understand spoken or signed language (i.e., receptive language) is relatively intact.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

The correct answer is: Developmental language disorder with impairment of mainly expressive language

Question **27**

Not answered

Marked out of 1

The risk factors common to children and adolescents who attempt suicide include all except

Select one:

- ☐ Recent life stressors
- ☐ Feelings of hopelessness
- ☐ Presence of impulsive traits
- ☐ Inability to tolerate negative affect
- ☐ Absence of cognitive distortions

Your answer is incorrect.

Explanation:

Adolescents who make cognitive errors (e.g. catastrophizing, personalisation) are at increased risk of attempting suicide.

Ref: Spirito A, Esposito-Smythers C, Wolff J, Uhl K. Cognitive-behavioral therapy for adolescent depression and suicidality. Child and Adolescent Psychiatric Clinics. 2011 Apr 1;20(2):191-204.

The correct answer is: Absence of cognitive distortions



Question 28

Not answered

Marked out of 1

Of those listed, which is the most common co-morbid psychiatric disorder seen in children with ADHD?

Select one:

- ☐ Bipolar disorder
- ☐ Sleep disorders
- ☐ Substance abuse
- ☐ Oppositional defiant disorder
- ☐ Conduct disorder

Your answer is incorrect.

Explanation:

50-80% of children with ADHD have a comorbid disorder. **Common comorbid conditions** include specific learning disorders (70%), autism spectrum disorder (59%), tic disorders/Tourette syndrome (55%), oppositional defiant disorder (30-50%) depression (12-50%), bipolar disorder (5-47%), anxiety disorders (15-35%), conduct disorder (3.5-10%), sleep disorders, and substance misuse.

Ref: Hill P. Attention-deficit hyperactivity disorder in children and adolescents: Assessment and treatment. BJPsych Advances. 2015 Jan;21(1):23-30.

Sample D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 668.

Gnanavel S et al. Attention deficit hyperactivity disorder and comorbidity: A review of literature. World Journal of Clinical Cases. 2019 Sep 6;7(17):2420.

The correct answer is: Oppositional defiant disorder

Question 29

Not answered

Marked out of 1

Which of the following is NOT a core diagnostic feature of ADHD?

Select one:

- ☐ Hyperactivity
- ☐ Language dysfunction
- ☐ Inattention
- ☐ Impulsivity
- ☐ None of the listed options

Your answer is incorrect.

Explanation:

ADHD is characterised by the three core symptoms of inattention, hyperactivity, and impulsiveness. Inattention includes being careless with detail, failing to sustain attention, appearing not to listen, failing to finish tasks, poor self-organisation, losing things, being forgetful, being easily distracted, and avoiding tasks requiring sustained attention. Hyperactivity is most evident in structured situations and includes fidgeting with hands or feet, leaving one's seat in class, running/climbing about, the inability to play quietly, and being 'always on the go'. Impulsiveness includes talking excessively, blurting out answers, the inability to wait one's turn, interrupting others, and intruding on others.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 668.

The correct answer is: Language dysfunction

Question 30

Not answered

Marked out of 1

Classically, children with enuresis have shown improvement with which one of the following antidepressants?

Select one:

- ☐ Fluoxetine
- ☐ Sertraline
- ☐ Imipramine
- ☐ Dothiepin
- ☐ Clomipramine

Your answer is incorrect.

Explanation:

Antidepressants, particularly imipramine, were the first medications used for enuresis, and are still used at times to take advantage of their anticholinergic side effects. However, imipramine does have a considerable side effect profile. Reboxetine, a norepinephrine reuptake inhibitor that has a non-cardiotoxic side effect profile, may be a safer alternative to the classically used imipramine in the treatment of childhood enuresis.

Ref: Caldwell PH et al. Tricyclic and related drugs for nocturnal enuresis in children. Cochrane Database of Systematic Reviews. 2016(1).

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 484.

The correct answer is: Imipramine

Question **31**

Not answered

Marked out of 1

Of the following, what is the common reason for parents and children to seek treatment for separation anxiety disorder?

Select one:

- ☐ Low mood
- ☐ Selective mutism
- ☐ Encopresis
- ☐ Nocturnal enuresis
- ☐ Excessive somatic complaints

Your answer is incorrect.

Explanation:

Worries in separation anxiety disorder often take the form of school refusal, fears and distress on separation, repeated complaints of physical symptoms such as headaches and stomachaches when separation is anticipated, and nightmares related to separation issues. Psychophysiological symptoms include stomachaches, headaches, nausea, vomiting, palpitations, and dizziness when anticipating separation.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 195-198.

The correct answer is: Excessive somatic complaints



Question 32

Not answered

Marked out of 1

In the very-early-onset variant of schizophrenia, the onset of symptoms is before age

Select one:

- ☐ 21
- ☐ 13
- ☐ 18
- ☐ 15
- ☐ 19

Your answer is incorrect.

Explanation:

Childhood-onset schizophrenia (COS; also called very-early-onset schizophrenia), a very rare and severe chronic psychiatric condition, is defined by an onset of positive symptoms (delusions, hallucinations, and disorganised speech or behaviour) before the age of 13. COS is associated with other neurodevelopmental disorders such as autism spectrum disorder and attention deficit and hyperactivity disorder. Copy number variations represent well documented neurodevelopmental disorder risk factors and, recently, de novo single nucleotide variations in genes involved in brain development have also been implicated in the complex genetic architecture of COS.

Ref: Fernandez A et al. Childhood-onset schizophrenia: a systematic overview of its genetic heterogeneity from classical studies to the genomic era. *Frontiers in Genetics*. 2019 Dec 18;10:1137.

Da Fonseca D, Fournier P. Very early onset schizophrenia. *L'encephale*. 2018 Dec 1;44(6S):S8-11.

The correct answer is: 13

Question **33**

Not answered

Marked out of 1

Heller syndrome is characterised by marked regression in several areas of functioning after at least how many years of normal development?

Select one:

- ☐ 2
- ☐ 5
- ☐ 3
- ☐ 8
- ☐ 4

Your answer is incorrect.

Explanation:

Childhood disintegrative disorder, also called Heller syndrome and disintegrative psychosis, was first described in 1908 as a deterioration over several months of intellectual, social, and language function occurring in 3- and 4-years-old with previously normal function. After the deterioration, the children closely resembled children with autistic disorder. Childhood disintegrative disorder is characterised by marked regression in several areas of functioning after at least 2 years of apparently normal development.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 103.

The correct answer is: 2



Question **34**

Not answered

Marked out of 1

A 15-year-old boy has moderate depression and no suicidality. He is a good student and has stable family support. The most suitable first-line treatment is

Select one:

- ☐ TCA antidepressants
- ☐ CBT alone
- ☐ Methylphenidate
- ☐ CBT + SSRIs
- ☐ SSRIs alone

Your answer is incorrect.

Explanation:

For the treatment of moderate-severe depression in young people aged 12-18 years, offer individual CBT for at least 3 months (if unsuitable, consider IPT for adolescents, family therapy, brief psychosocial intervention, or psychodynamic psychotherapy). Consider combination treatment (fluoxetine and psychological therapy) for initial treatment as an alternative to psychological therapy.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the identification and management of depression in children and young people. 2019.

The correct answer is: CBT alone

Question 35

Not answered

Marked out of 1

Which of the following scales could be used in epidemiological studies as a screening tool for caseness among school-going children before further diagnostic procedures?

Select one:

- ☐ Social Communication Questionnaire
- ☐ Connors Rating Scale
- ☐ General Health Questionnaire
- ☐ Strengths and Difficulties Questionnaire
- ☐ Child Behavioural Checklist

Your answer is incorrect.

Explanation:

The Strengths and Difficulties Questionnaire is a brief emotional and behaviour screening questionnaire for children and young people that can capture the perspective of children and young people, their parents and teachers. Subscales include: emotional symptoms, conduct problems, hyperactivity/inattention, peer relationships problem, and prosocial behaviour.

Ref: <https://www.corc.uk.net/outcome-experience-measures/strengths-and-difficulties-questionnaire/>

The correct answer is: Strengths and Difficulties Questionnaire

Question **36**

Not answered

Marked out of 1

The onset of autistic spectrum disorder occurs generally before the age of

Select one:

- ☐ Two
- ☐ Four
- ☐ Five
- ☐ Three
- ☐ Eight

Your answer is incorrect.

By definition, the onset of the autistic disorder is before the age of 3 years, although, in some cases, it is not recognized until a child is much older. Autistic disorder is four to five times more frequent in boys than in girls

The correct answer is: Three



Question **37**

Not answered

Marked out of 1

Miss. X is a 16-year-old girl who was assessed in the CAMHS outpatient clinic and diagnosed with bulimia nervosa. Which of the following psychotherapeutic interventions is the most effective treatment in this age group?

Select one:

- ☐ Family therapy
- ☐ Behavioural therapy
- ☐ Psychodynamic psychotherapy
- ☐ CBT
- ☐ Interpersonal therapy

Your answer is incorrect.

Explanation:

Adolescents with bulimia nervosa may be treated with CBT adapted, as needed, to suit their age, circumstances, and level of development, and including the family as appropriate. NICE advises considering bulimia-nervosa-focused guided self-help (using cognitive behavioural materials) supplemented with brief supportive sessions. If this is unacceptable or ineffective after 4 weeks, consider individual CBT-ED for bulimia nervosa.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 693.

National Institute for Health and Clinical Excellence. NICE guideline Eating Disorders: recognition and treatment 2017.

The correct answer is: CBT

Question 38

Not answered

Marked out of 1

Which of the following sleep-wake disorders is associated with the use of stimulants?

Select one:

- ☐ REM sleep behaviour disorder
- ☐ Periodic limb movement disorder
- ☐ Sleep-related bruxism
- ☐ Central hypersomnia
- ☐ Sleep terrors

Your answer is incorrect.

Explanation:

Sleep-related bruxism is characterised by clenching and grinding of the teeth during sleep. It may result in temporomandibular joint pain, wearing down of the teeth, and/or injury to the tongue or mouth. It is most common at the transition to sleep, in stage 2 sleep, and during REM sleep. It may be related to stress and/or anxiety, dentition abnormalities, sleep-related breathing disorders, the use of stimulants, alcohol ingestion, and treatment with some SSRIs.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 476.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 501-502.

The correct answer is: Sleep-related bruxism

Question 39

Not answered

Marked out of 1

Of those listed, the most common blood abnormality in adolescents with anorexia nervosa is

Select one:

- ☐ Low mean corpuscular haemoglobin
- ☐ Macrocytosis
- ☐ Microcytosis
- ☐ Normocytic normochromic anaemia
- ☐ Low hemoglobin concentration

Your answer is incorrect.

Explanation:

Anaemia, leukopenia, and although less frequently, thrombocytopaenia are possible haematological complications of anorexia nervosa (AN) considered strictly secondary to chronic malnutrition. Usually, anaemia in AN is normocytic and normochromic, and leukopenia is characterised by lymphocytopenia or neutropenia; thrombocytopaenia may also be present.

Ref: De Filippo E et al. Hematological complications in anorexia nervosa. European Journal of Clinical Nutrition. 2016 Nov;70(11):1305-8.

The correct answer is: Normocytic normochromic anaemia

Question 40

Not answered

Marked out of 1

Report cards may be used as a tool in the treatment of which of the following disorders?

Select one:

- ☐ School refusal
- ☐ ADHD
- ☐ Premenstrual dysphoria
- ☐ Adolescent depression
- ☐ Tic disorders

Your answer is incorrect.

Explanation:

Research suggests that daily report cards are effective in treating a range of ADHD symptoms and improving school outcomes, including academic achievement in some cases. This intervention typically involves teachers evaluating a student's behaviour at school against pre-determined targets, and parents subsequently providing reinforcement at home for positive reports.

Ref: Moore DA et al. Daily report cards as a school-based intervention for children with attention-deficit/hyperactivity disorder. Support for Learning. 2016 Feb;31(1):71-83.

The correct answer is: ADHD



Question **41**

Not answered

Marked out of 1

Despite early behavioural interventions, the symptoms in autism spectrum disorder that tend not to improve over time are related to

Select one:

- ☐ Activities of daily living
- ☐ Ritualistic and repetitive behaviours
- ☐ Impairments in social interaction
- ☐ Intellectual functioning
- ☐ Impairments in communication

Your answer is incorrect.

Explanation

Autism spectrum disorder is typically a lifelong, albeit heterogeneous disorder with highly variable severity and prognosis. Early intensive behavioural interventions can have a profoundly positive impact (in some cases, can lead to recovery and function in the average range). The symptoms that do not seem to improve substantively over time with early behavioural interventions are related to ritualistic and repetitive behaviours. Specifically, repetitive sensorimotor behaviours appear to be common, consistent, and potentially severe.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 127.

International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

The correct answer is: Ritualistic and repetitive behaviours

Question **42**

Not answered

Marked out of 1

Which of the following is included in the ICD-11 description of autism spectrum disorder?

Select one:

- ☐ A particular interest in collecting things
- ☐ Difficulty with mathematics
- ☐ Rigidity of thought
- ☐ Intelligence quotient below 70
- ☐ Limitations in the sharing of interests with others

Your answer is incorrect.

Explanation:

According to ICD-11, essential features of autism spectrum disorder include persistent deficits in initiating and sustaining social communication and reciprocal social interactions that are outside the expected range of functioning given the individual's age and level of intellectual development. Manifestations may include limitations in the mutual sharing of interests with others.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

The correct answer is: Limitations in the sharing of interests with others



Question 43

Not answered

Marked out of 1

Which of the following symptoms is rare in early-onset schizophrenia?

Select one:

- ☐ Hallucinations
- ☐ Negative symptoms
- ☐ Thought disorder
- ☐ Changes in affect
- ☐ Catatonia

Your answer is incorrect.

Explanation:

Prodromal symptoms of early-onset schizophrenia can include odd ideas, eccentric interests, change in affect, and unusual or bizarre perceptual experiences. Early-onset schizophrenia is characterised by a more insidious onset, negative symptoms, greater disorganisation (incoherence of thought and disordered sense of self), hallucinations in different modalities and, for relatively fewer patients, systematised or persecutory delusions. Catatonia is seldom seen.

Ref: Hollis C. Schizophrenia in children and adolescents. BJPsych Advances. 2015 Sep;21(5):333-41.

Harvey PD, Isner EC. Cognition, social cognition, and functional capacity in early-onset schizophrenia. Child and Adolescent Psychiatric Clinics. 2020 Jan 1;29(1):171-82.

The correct answer is: Catatonia

Question **44**

Not answered

Marked out of 1

What is the medication of choice for an 8-year-old child with ADHD and tics?

Select one:

- ☐ Methylphenidate
- ☐ Beta-blocker
- ☐ Dexamphetamine
- ☐ Antipsychotic
- ☐ Noradrenaline reuptake inhibitor

Your answer is incorrect.

Explanation:

Atomoxetine is a norepinephrine reuptake inhibitor commonly prescribed for ADHD in children over 6 years of age. Unlike stimulants approved for ADHD, atomoxetine does not have abuse potential and may not exacerbate co-morbid tics. It can be given once a day in the morning.

Ref: Stahl SM. Stahl's Essential Psychopharmacology: Prescriber's Guide, 7th Edition. 2021. pp. 79-83.

The correct answer is: Noradrenaline reuptake inhibitor



Question 45

Not answered

Marked out of 1

Which of the following is the best treatment option for a teenager with ADHD and ongoing substance use?

Select one:

- ☐ Imipramine
- ☐ Pemoline
- ☐ Methamphetamine
- ☐ Methylphenidate
- ☐ Atomoxetine

Your answer is incorrect.

Explanation:

Atomoxetine is a norepinephrine reuptake inhibitor commonly prescribed for ADHD. Unlike stimulants approved for ADHD, atomoxetine does not have abuse potential and may be useful where stimulant diversion is a problem.

Ref: Stahl SM. Stahl's Essential Psychopharmacology: Prescriber's Guide, 7th Edition. 2021. pp. 79-83.

Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 574.

The correct answer is: Atomoxetine



Question 46

Not answered

Marked out of 1

Asperger syndrome is characterised by all of the following except

Select one:

- ☐ Impaired awareness of the feeling of others
- ☐ Restricted interests
- ☐ Impaired intellectual ability and syntactical speech
- ☐ Repetitive behaviour patterns
- ☐ Impaired reciprocal social interactions

Your answer is incorrect.

Explanation:

Asperger syndrome was first described by Hans Asperger in 1944, but only eponymously named in 1981 by Lorna Wing. It is characterised by severe persistent impairment in reciprocal social interactions, repetitive behaviour patterns, and restricted interests. There is impairment in social relatedness, as well as repetitive and stereotyped patterns of behaviour, without delay or marked aberrant language development and usage. Cognitive abilities and significant adaptive skills are age-appropriate, although social communication is impaired. IQ and language are normal or, in some cases, superior.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 820.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 122.

The correct answer is: Impaired intellectual ability and syntactical speech

Question **47**

Not answered

Marked out of 1

In early childhood, depression will manifest as clingy behaviour, school refusal and exaggerated fears. The percentage of adolescents up to 19 years of age who will have experienced a diagnosable episode of depression in the Europe is

Select one:

- ☐ 3%
- ☐ 40%
- ☐ 20%
- ☐ 1%
- ☐ 5%

Your answer is incorrect.

Around 20-30% can have a diagnosable episode before age 19, but often these episodes are unattended and appear only in epidemiological surveys seeking lifetime prevalence

The correct answer is: 20%



Question **48**

Not answered

Marked out of 1

The most common adverse outcome of conduct disorder is the development of which of the following conditions in adulthood?

Select one:

- ☐ Antisocial personality disorder
- ☐ Oppositional defiant disorder
- ☐ Eating disorders
- ☐ Paraphilic disorders
- ☐ Psychosis

Your answer is incorrect.

Explanation:

Around half of children with conduct disorder will receive a diagnosis of antisocial personality disorder as adults.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 665.

The correct answer is: Antisocial personality disorder



Question **49**

Not answered

Marked out of 1

The common presentations of anxiety disorders in preschool children include

Select one:

- ☐ Recurrent arguments
- ☐ Hypochondriacal fretting
- ☐ Clinging behaviour
- ☐ Somatic complaints
- ☐ Aggressive behaviour

Your answer is incorrect.

Anxiety symptoms vary with age. Tearfulness and clinging in preschool children. Somatic complaints and hypochondriacal fretting are common in middle childhood (5-12) as are irritability and aggressive behaviour. Separation anxiety disorder and specific phobia usually have onset in early childhood.

The correct answer is: Clinging behaviour



Question **50**

Not answered

Marked out of 1

Which of the following disorders is among the most common psychiatric presentations seen in the adolescents?

Select one:

- ☐ Personality disorders
- ☐ Depressive disorders
- ☐ Psychotic disorders
- ☐ Anxiety disorders
- ☐ Alcohol and substance misuse disorders

Your answer is incorrect.

Anxiety disorders are among the most common conditions affecting children and adolescents, with prevalence in the 4% to 20% range. They adversely impact self-esteem, social relationships, and academic performance.

The correct answer is: Anxiety disorders



Question **51**

Not answered

Marked out of 1

Select the adolescent-onset disorder for which social disadvantage is not an established risk factor.

Select one:

- ☐ Unipolar depression
- ☐ Substance use disorders
- ☐ Anorexia
- ☐ Schizophrenia
- ☐ Alcohol dependence

Your answer is incorrect.

Anorexia is commoner among upper social class, but social class bias may be modest, and social disadvantage is not a strong determinant (Kendell 1973)

The correct answer is: Anorexia



Question 52

Not answered

Marked out of 1

Which of the following is least likely to be associated with PANDAS?

Select one:

- ☐ Auditory hallucinations
- ☐ Anxiety
- ☐ Tics
- ☐ Obsessions
- ☐ Low mood

Your answer is incorrect.

Explanation:

Paediatric autoimmune neuropsychiatric disorder associated with streptococcal infections (PANDAS) is characterised by the sudden and dramatic onset of obsessive-compulsive disorder, tic symptoms, or severely restricted food intake and neuropsychiatric comorbidity, also of sudden and severe onset, following group A streptococci (GAS) infection. Common comorbidities include anxiety (including separation anxiety); emotional lability and/or depression; irritability, aggression and/or severely oppositional behaviours; behavioural (developmental) regression; deterioration in school performance (ADHD-like symptoms, memory deficits, cognitive changes); sensory or motor abnormalities; somatic signs and symptoms (including sleep disturbances, enuresis, or increased urinary frequency). Nearly 1/3 of children endorse concerns related to suicide. The term Pediatric Acute-onset Neuropsychiatric Syndrome (PANS) is used when the etiology or mechanism of the abrupt onset is unknown.

Ref: Warrilow A, Morton M. Autoimmune disorders in child psychiatry: keeping up with the field. BJPsych Advances. 2015 Nov;21(6):367-76.

Cabrera-Mendoza B et al. Diagnosis in PANDAS: An Update. Current Psychiatry Research and Reviews Formerly: Current Psychiatry Reviews. 2019 Dec 1;15(4):237-47.

The correct answer is: Auditory hallucinations

Question 53

Not answered

Marked out of 1

Seizures and irregular respiration are common features of

Select one:

- ☐ Asperger syndrome
- ☐ ADHD
- ☐ Rett syndrome
- ☐ Childhood-onset schizophrenia
- ☐ Tourette syndrome

Your answer is incorrect.

Explanation:

Rett Syndrome:

- Believed to be caused by dominant X-linked gene
- Degenerative disorder that almost exclusively affects females
- Initially normal development
- Ages 1-4 yrs: purposeful hand movements & spoken language lost; stereotypical hand movements (hand-wringing) & autism
- Ataxia, facial grimacing, teeth-grinding, and loss of speech
- Irregular respiration with episodes of hyperventilation, apnea, and breath-holding
- Progressive gait disturbance, scoliosis, and seizures
- Sleep disturbances are common; initial and/or intermediate insomnia, leading to excessive daytime sleep

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 103.

Persico AM et al. The psychopharmacology of autism spectrum disorder and Rett syndrome. In Handbook of clinical neurology 2019 Jan 1 (Vol. 165, pp. 391-414). Elsevier.

The correct answer is: Rett syndrome



Question **54**

Not answered

Marked out of 1

In the UK, which of the following drugs should be prescribed first in the treatment of obsessive-compulsive disorder in childhood?

Select one:

- ☐ Fluoxetine
- ☐ Clomipramine
- ☐ Sertraline
- ☐ Paroxetine
- ☐ Fluvoxamine

Your answer is incorrect.

Explanation:

Sertraline (from age 6 years) and fluvoxamine (from age 8 years) are the selective serotonin reuptake inhibitors (SSRIs) licensed in the UK for the treatment of OCD in young people. Studies spanning 20 years have established the efficacy of SSRIs in the paediatric population in placebo-controlled trials. Sertraline and fluoxetine are equally effective, but fluvoxamine may be less so. Paroxetine is not recommended for use in children and young people. On occasion, medications other than sertraline and fluvoxamine may be used as 'off-label' preparations with suitable caution. NICE guidance for the treatment of OCD recommends the use of SSRIs before the use of clomipramine, because of clomipramine's propensity for side effects and need for cardiac monitoring.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 561-562.

National Institute for Health and Clinical Excellence. The NICE guideline on core interventions in the treatment of obsessive-compulsive disorder and body dysmorphic disorder. Updated 2020.

The correct answer is: Sertraline

Question 55

Not answered

Marked out of 1

The onset of oppositional defiant disorder is generally between the ages of

Select one:

- ☐ 5 and 10 years
- ☐ 12 and 15 years
- ☐ 3 and 8 years
- ☐ 10 and 15 years
- ☐ 2 and 5 years

Your answer is incorrect.

Explanation:

Although ODD can begin as early as **3 years of age**, it typically is noted by **8 years of age** and usually not later than early adolescence.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 189.

The correct answer is: 3 and 8 years



Question **56**

Not answered

Marked out of 1

The chief differential medical problem to be excluded in children with encopresis is

Select one:

- ☐ Vitamin B 12 deficiency
- ☐ Diverticulitis
- ☐ Down syndrome
- ☐ Hirschsprung disease
- ☐ Rectal carcinoma

Your answer is incorrect.

Explanation:

The chief differential medical problem to be excluded in children with encopresis is aganglionic megacolon or Hirschsprung disease, in which a patient may have an empty rectum and no desire to defecate but may still have an overflow of feces. The disorder occurs in 1 in 5000 children; signs appear shortly after birth. Faulty nutrition is rarely the cause of encopresis with constipation and overflow incontinence. Other conditions are rare as well, including structural disease of the anus, rectum, and colon, adverse drug effects, or non-gastrointestinal medical (endocrine or neurologic) disorders.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 482.

The correct answer is: Hirschsprung disease

Question **57**

Not answered

Marked out of 1

Known risk factors for child abuse include all except

Select one:

- ☐ Young, immature parents
- ☐ A parent figure who is biologically unrelated to the child
- ☐ Being a first-born child
- ☐ High socio-economic class
- ☐ A parent who was abused as a child

Your answer is incorrect.

Explanation:

Individual poverty is a well-known risk factor for child maltreatment. An association between socioeconomic status and the risk of child maltreatment has been demonstrated based on local, neighbourhood, and larger-scale studies.

Ref: Imran S et al. Association between socioeconomic status and risk of hospitalization due to child maltreatment in the USA. Journal of investigative medicine. 2019 Feb 1;67(2):346-9.

The correct answer is: High socio-economic class



Question 58

Not answered

Marked out of 1

Melatonin is considered an effective treatment for children with insomnia and learning difficulties. Which of the following significantly improves on treatment with this drug?

Select one:

- ☐ Number of night awakenings
- ☐ Sleepwalking
- ☐ Sleep apnoea
- ☐ Early morning awakening
- ☐ Sleep onset latency

Your answer is incorrect.

Explanation:

All forms of insomnia are more common in children with learning difficulties, autism, ADHD, and sensory impairments (particularly visual). Although behavioural interventions should be the primary intervention and have a robust evidence base, exogenous melatonin is now the 'first-line' medication prescribed for childhood insomnia. Melatonin has been shown to decrease sleep onset latency, increase total sleep time, and improve overall sleep quality. The effects of melatonin on sleep are modest but do not appear to dissipate with continued melatonin use.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 593-594.

The correct answer is: Sleep onset latency

Question 59

Not answered

Marked out of 1

What is the UK prevalence for ADHD in children?

Select one:

- ☐ 3-5%
- ☐ 1-2%
- ☐ 7-10%
- ☐ 0.25%
- ☐ 10-15%

Your answer is incorrect.

Explanation:

The UK prevalence for ADHD is just under 5%; 3-4% in boys and just under 1% in girls (four times more common in boys).

Ref: Hill P. Attention-deficit hyperactivity disorder in children and adolescents: Assessment and treatment. BJPsych Advances. 2015 Jan;21(1):23-30.

The correct answer is: 3-5%



Question 60

Not answered

Marked out of 1

Which of the following pharmacological options is suitable for the treatment of a girl with ADHD and frequent aggressive outbursts?

Select one:

- ☐ Pemoline
- ☐ Clonidine
- ☐ Olanzapine
- ☐ Imipramine
- ☐ Methylphenidate

Your answer is incorrect.

ADHD and aggression: Atomoxetine unlikely beneficial; consider behavioural therapy; consider atypical antipsychotics, clonidine

The correct answer is: Olanzapine



Question **61**

Not answered

Marked out of 1

What proportion of 15 to 16-year-old adolescents admit to incidents of deliberate self-harm in the UK?

Select one:

- ☐ 14.9%
- ☐ 8.9%
- ☐ 22.9%
- ☐ 12.9%
- ☐ 6.9%

Your answer is incorrect.

Actual incidents of DSH are admitted by only 6.9% of adolescents. The prevalence of ideation is estimated to be far higher.

The correct answer is: 6.9%



Question **62**

Not answered

Marked out of 1

Which one of the following is not categorised as a pervasive developmental disorder?

Select one:

- ☐ ADHD
- ☐ Autism
- ☐ Childhood disintegrative disorder
- ☐ Rett's syndrome
- ☐ Asperger's syndrome

Your answer is incorrect.

According to DSM-IV, PDDs include autism, Asperger's syndrome, Rett's syndrome, Childhood disintegrative disorder and PDD-Not Otherwise Specified. DSM-5 has dropped Asperger's syndrome, which is now brought under autistic spectrum disorders.

The correct answer is: ADHD



Question 63

Not answered

Marked out of 1

Which of the following statements regarding enuresis is true?

Select one:

- ☐ Antidiuretic hormone worsens enuresis
- ☐ Most children remit over time in childhood
- ☐ Behavioural treatments have no evidence base
- ☐ The diagnosis can be made only after age 3
- ☐ There is no familial aggregation

Your answer is incorrect.

Explanation:

Enuresis is the repeated voiding of urine into a child's clothes or bed; the voiding may be involuntary or intentional. For the diagnosis to be made, a child must exhibit a developmental or chronological age of at least 5 years. Genetic factors play a role in the expression of enuresis, given that the emergence of enuresis is significantly higher in first-degree relatives. A child's risk for enuresis is more than seven times higher if their father was enuretic. Enuresis is often self-limited, and a relatively high rate of spontaneous remission occurs over time in childhood. A classical conditioning behavioural intervention with the bell (or buzzer) and pad (alarm) apparatus is generally the most effective treatment for nocturnal enuresis. Desmopressin, an antidiuretic compound that is available as an intranasal spray, has shown success in reducing enuresis.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 483-485.

The correct answer is: Most children remit over time in childhood



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